

Seyfried diet — for Kim, to keep

For Kim, about Greg · 28 August 2026 · One letter, not a protocol

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Hi Kim,

This replaces the texts — keep this copy.

I wanted the diet piece in one simple letter because I know I sent you a lot. Same takeaway as before, written so you can print it, mail it, or stick it on the fridge.

One line: do not replace carbs with huge amounts of meat. Fat is the main calorie source. Protein adequate / moderate — enough to protect muscle.

The Seyfried / metabolic approach is **not a high-protein carnivore diet**. Carbs and sugar stay very low. Protein stays moderate so it is not the main calorie source, but high enough to protect muscle during treatment. Fat carries most of the calories.

FOR GREG, IN PLAIN TERMS

High-grade neuroendocrine / small-cell, starting in the sinonasal area. Treatment has started. Smell, taste, and appetite may be limited because of the sinonasal / olfactory-groove involvement — soft, fatty liquids will often matter more than steak. He also has liver and bone mets, so calories and muscle matter. Do not chase a “perfect keto” day if he is not eating. Add extra fats only if he tolerates them; chemo can bring nausea or loose stools.

FRIDGE LIST

- **Carbs / sugar:** very low.
- **Protein:** moderate, and enough to protect muscle. Prefer whole-food when he can tolerate it — eggs, fish, clean meats — paired with fat. Not huge portions.
- **Fat (main calories):** olive oil through the day, on whatever he can eat. Butter in bone broth and in tea if tolerated. Avocado, MCT oil, ghee.
- **Bone broth:** when eating is hard. Add butter if he tolerates it — more fat, not more protein or carbs.
- **Shake only if needed:** if he simply cannot get enough food, a clean unsweetened low-carb pea or rice protein. Not a first choice.

GLUTAMINE — ONE CLARIFICATION

The body makes glutamine itself. Do not starve protein to “control glutamine.” Keeping protein moderate is so it is not the bulk of his calories — not because cutting protein will switch off glutamine. We do not want him losing muscle because we restricted protein too hard.

GKI, SIMPLY

GKI means Glucose-Ketone Index. It is just the relationship between blood glucose and ketones. Checking both gives us something objective, instead of assuming a food is “keto.” Useful when he is eating. Not a contest, and not more important than food going in.

If weight or muscle drops, reassess protein and calories. Do not rigidly restrict. Eating and holding muscle come first.

Dr. Max fills in the numbers — exact protein and total calories from Greg’s weight, appetite, treatment, and whether he is maintaining muscle. This letter is the frame, not those targets.

Don’t replace carbs with huge amounts of meat. Replace most of those calories with healthy fat, keep protein adequate / moderate, and protect his muscle. I’m here if any of this is unclear. Keep this copy.

Family caregiver documentation of the Seyfried / metabolic diet framing already sent by text. Not medical advice. Not a start-everything protocol. Diet only — no drugs, supplements, or melatonin on this page. Treatment decisions stay with Greg’s clinicians.